جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : عزه عبدالعليم عيسي عبدالعال
المحافظة : الجيزه **الحي / المدينة :** امبابه **العنوان :** ~~
رقم البطاقة : 26507150103083 **رقم التليفون :** 01147226212 **تاريخ تسجيل الإستمارة :** 10-10-2026
العمر(تاريخ الميلاد) : 15-07-1965 **المهنة :** بدون عمل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : علاج مبادرة سرطان الثدي تراستوزوماب تحت الجلد و بيرتوزوماب(فيسجو) [14117] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: عزه عبدالعليم عيسي عبدالعال
National ID: 26507150103083

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	**بشأن تقرير طبي للمريض**
بقسم:	**رقم طبي:**

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	**تاريخ وساعة الخروج :** PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي Phesgo) Subcutaneous trastuzumab and pertuzumab)

التاريخ : 05-07-2026	**الوقت :** -- --:--
	الطبيب المعالج : د/شيماء لاشين





National ID – Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

جوستافا روسي

مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

08/03/2026

MDT FORM

Name:	Essia Abd Elaal	MRN:	10829
Age:	61 Y	ID:	26507150103083
Menopausal Sta		Phone:*	01129394464
NRS SCORE:		Marital Status:	MARRIED
BMI:		Family History:	NEG
Gender			
CO Morbidity:	HTN CARDIAC DM	Reason Of MDT:	NEW CASE
Surgical History:*	CATHETER OPEN CHOLYCYSTECTOMY HAEMORRODECTOMY	ECOG Performance Status	◉ 0 ○ 1 ○ 2 ○ 3 ○ 4

Clinical Data:

BX: LT ILC PLEOMORPHIC G2

ER: 8/8 PR: 8/8 HER2: +2 KI67: SISH AM

History:

Sonomammography:
*SM : ACR B
LT SPECULATED MASS IN UOQ 39*38 MM AT 3 O'CLOCK WITH SKIN DIMPLING AND NIPPLE RETRACTION
LT PATHOLOGICAL LN 39*26 MM

Biomarker/Molecular:

Mets Workup:
CT CAP FREE
BONE SCAN FREE

Recommendations:
FOR CLIP INSERTION THEN NACT IF WE PLANNED TO DO BCS
AR REGARD CLINICAL EXAMINATION T2N2M0 WITH AVERAGE BREAST SIZE
MASS CAUSING DIMPLING NOT INFILYRATE THE SKIN

MDT Outcome & Plan:
FOR CESM(ILC + SM FROM 4 /2026)PRE NACT EVALUATE CONTRALATERAL SIDE AND SURGICAL DECISION
,ECHO, PORTA CATH
START TCHP X 6CYCLES
THEN LT MRM(SKIN DIMPLING AND NIPPLE RETRACTION)



TNM

	Right Side				Left Side			
	Q1	Q2	Q3	Q4	Q1	Q2	Q3	Q4

Clinical Staging:

Pathological Staging:

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY

MDT Form
[[DOCTYPE:MDT_FORM]]

Name: عزه عبدالحليم عيسى عبدالعال **MR. :** 10829	

Detailed Medical Report

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من		**بشأن تقرير طبي للمريض**	
بقسم:		**رقم طبي:**	

ننشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول :		**تاريخ وساعة الخروج :**	

وبعد الكشف والفحوصات والمتابعة وجد انه

The patient is a 61-year-old individual with hypertension, cardiac disease, and diabetes mellitus. There is no reported family history of malignancy. The clinical history includes a left-sided invasive lobular carcinoma, pleomorphic, grade 2.

Biopsy of the left breast revealed invasive lobular carcinoma, pleomorphic, grade 2. Immunohistochemistry shows estrogen receptor 8/8, progesterone receptor 8/8, HER2 +2, and Ki-67 reported as SISH amplification. Imaging demonstrates a left-sided speculated mass in the upper outer quadrant measuring 39 × 38 mm at the 3 o'clock position with overlying skin dimpling and nipple retraction, classified as ACR B on sonomammography, and a left pathological lymph node measuring 39 × 26 mm. Staging workup with CT of the chest, abdomen, and pelvis and bone scan are negative for metastatic disease.

The referring team recommends clip insertion followed by neoadjuvant chemotherapy if breast-conserving surgery is planned, noting a clinical stage of T2 N2 M0 with average breast size and a mass causing dimpling without skin infiltration. The multidisciplinary team advises contrast-enhanced spectral mammography (CESM) for the left invasive lobular carcinoma and surgical mammography from April 2026, pre-neoadjuvant chemotherapy evaluation of the contralateral side, and further surgical decision-making, together with echocardiography and cardiac catheterization. The plan is to initiate TCHP chemotherapy for six cycles, then proceed with left mastectomy given the skin dimpling and nipple retraction.

تحريرا في ,

التاريخ :	23-09-2026	**الوقت :**	

الطبيب المعالج :

يعتمد ,

جوستافروسي انترناشونال

مبادرة رئيس الجمهورية لدعم صحة المرأة المصرية

البنت المصرية هي صحة مصر

MDT FORM 2016 /2026 G

Name	عزة عبد الحليم	MRN	1089
		ID	
Age		Phone	01129944644
Menopausal status	Post	Marital status	married
NRS SCORE		Family history	—
BMI		REASON OF MDT	New
CO morbidity	HTN, DM		

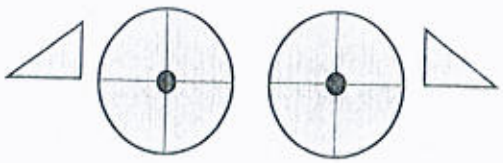
Clinical Data

BX: ILC

ER:	PR:	HER 2:	Ki67:
8/8	8/8	amplified	10%

Sonomammography: Lt UOQ 3.6x3.7cm

R L



TNM

Mets Workup:

Free

Bone Scan → Requested

Recommendations:

CESM

MDT OUTCOME & PLAN

CESM

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
	د. أحمد				

MDT Form
[[DOCTYPE:MDT_FORM]]

Name : Azza Abdelhalim Eissa Abdel Aal **Date : 23-07-2026 07:42**
Branch : Dokki **Medical No. : 423282**
Ref.Physician Prof : Goustav Roussey Egypt

WHOLE-BODY BONE SCINTIGRAPHY REPORT

Indication: Newly diagnosed left breast carcinoma. Staging workup.

PROCEDURE

Anterior and posterior whole-body images following IV ^{99m}Tc-MDP.

FINDINGS

- No definitive focal active or cold osseous lesions could be depicted that would account for metastatic pathology.
- Variable tracer uptake along spine and sternum consistent with degenerative changes.
- Mild-to-moderate uptake in both shoulders and knees bilaterally—consistent with degenerative joint disease.
- Normal renal excretion with bladder activity. No abnormal extraskeletal uptake.

IMPRESSION

- No scintigraphic evidence of skeletal metastases.
- Follow up as per clinical pathway.

Much oblige
Dr. Alaa Nagi, MD
Nuclear medicine

NILE DIAGNOSTIC CENTER

Bone Scan
[[DOCTYPE:BONE_SCAN]]

GUSTAVE ROUSSY
INTERNATIONAL

Patient Name	: الأستاذه / عزه عبد الحليم عيسى	Request No.	: 13006082
Patient No.	: 265407150103083	Request Date	: 23/04/2026
Path No.	: 1157/26	Verification Date	: 11/05/2026
Sex	: Female	Financial Category	: جوستاف روسي - مبادره
Age	: 60 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: .marwa mohamed youss

Pathology Report

Clinical Data: Left breast UOQ mass.(BIRADs 5).
Nature of specimen: Tru-cut biopsy, collected on 23/4/2026, at 12:20 pm, cold ischemia time is 20 seconds.

Gross examination:
Two gray white tissue cores, each measured 1 cm long. Totally submitted.

Microscopic examination
* Histological type: Invasive pleomorphic lobular carcinoma.
* Grade 2 (differentiation 3, nuclear pleomorphism 2, mitosis 2).
* Ductal carcinoma in situ: Absent.
* Lobular carcinoma in situ: Absent.
* Angioinvasion: Absent.
* Perineural invasion: Absent.

Diagnosis
Left breast UOQ mass, Tru-cut biopsy:
Invasive pleomorphic lobular carcinoma, Grade 2.
- For further IHC (ER, PR, HER2, Ki67).

Signature
Dr. Walaa Ibrahim Ghoneim.

Verified by :
Dr.Duaa Mohamed

CAP ACCREDITED
غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

USTAVE/
OUSSY
TERNATIONAL

Patient Name	: الأستاذه / عزه عبد الحليم عيسى	Request No.	: 13006082
Patient No.	: 265407150103083	Request Date	: 23/04/2026
Path No.	: 1157/26	Verification Date	: 11/05/2026
Sex	: Female	Financial Category	: جوستاف روسي - مبادره
Age	: 60 Y	Department	: Out Patient
Referreing Doc.	:	IHC No.	:
		Registered by	: .marwa mohamed you

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Left breast UOQ mass (BIRADs 5) diagnosed "Invasive pleomorphic lobular carcinoma, grade 2".
Nature of specimen: Tru-cut biopsy, collected on 23/4/2026, at 12:20 pm, cold ischemia time is 20 seconds.

Technique:
- Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) stained by rabbit monoclonal antibodies using Ultraview HRP DAB as chromogen and hematoxylin as counter-stain. The sections were treated against: ER clone (1E2), PR clone (SP1), Her2 clone (4B5), Ki67 clone (30-9) & SISH.

Results:
* IPT (All controls showed appropriate reactivity):
- ER: Strong nuclear reaction in 95% of tumor cells (Score 8/8).
- PR: Strong nuclear reaction in 95% of tumor cells (Score 8/8).
- Her2: Moderate complete membrane reaction in more than 10% of tumor cells (Score 2+).
- SISH: Amplified.
(Average Her2 copy No.: (4.3), Her2/CEP17 ratio: (2.09)).
- Ki67%: 10%.

Results:
Left breast mass, Tru-cut biopsy:
IPT:
- ER: Positive (score 8/8).
- PR: Positive (score 8/8).
- Her2: Equivocal (score 2+).
- SISH: Amplified, Group I.

Verified by :
Dr.Duaa Mohamed

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Immunohistochemistry
[[DOCTYPE:IHC]]

USTAVE/
OUSSY
ERNATIONAL

Patient Name	الأستاذه / عزه عبد الحليم عيسى	Request No.	13006082
Patient No.	265407150103083	Request Date	23/04/2026
Path No.	1157/26	Verification Date	11/05/2026
Sex	Female	Financial Category	جوستاف روسي - مبادره
Age	60 Y	Department	Out Patient
Referreing Doc.		Registered by	.marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

4 but less than 6. (Recounting give the same result).
• Group V (Negative): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is less than 4.

Signature
Dr. Walaa Ibrahem Ghoneim.

Verified by :
Dr.Duaa Mohamed

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

CAP ACCREDITED

Immunohistochemistry
[[DOCTYPE:IHC]]

جوستاف روسي

Patient's Name:	**Azza Abdelhalim Eissa Abdel Aal**
Date:	**2026-06-09**
Referred by:	**Prof.Dr. no doctor**
Hospital:	**Goustave Roussey International Egypt**

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST WITH CORONAL RECONSTRUCTION: -

Clinical History:

- **A new case of left breast cancer.**

Findings:

- Left breast UOQ mass is seen measuring 3.6x3.7cm.
- Left axillary enlarged LNs largest 3.5cm.
- Clear right breast and right axilla.
- Clear both lung fields. No sizable pulmonary nodules, masses, areas of consolidation or cavitary lesions.
- No hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

OPINION:

- Left breast UOQ mass.
- Left axillary enlarged LNs.
- Clear right breast and right axilla.
- No lung metastasis.

MULTI-SLICE CT SCAN OF THE ABDOMEN AND PELVIS AFTER I.V. CONTRAST ADMINISTRATION: -

Findings:

- The liver is of average size with homogenous CT texture. No focal lesions or contour irregularities.

CT Scan
[[DOCTYPE:CT_SCAN]]

Patient's Name:	**Azza Abdelhalim Eissa Abdel Aal**
Date:	**2026-06-09**
Referred by:	Prof.Dr. **no doctor**
Hospital:	**Goustave Roussey International Egypt**

- Patent homogenously enhancing portal vein and is main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

Opinion:

- **No hepatic or bony metastasis.**

Dr. Mai Shaaban MD

CT Scan
[[DOCTYPE:CT_SCAN]]

Ministry of Health and Population

Specialized Medical Centers

Nasser Institute Hospital

وزارة الصحة و السكان

أمانة المراكز الطبية المتخصصة

معهد ناصر للبحوث و العلاج

الاســـــم: عزة عبدالحليم عيسى

التاريـــــخ: 2026/4/4

Clinical Background

Diagnostic mammography for a 60 y/o female complaining of left breast mass. Negative family history.

Bilateral Digital soft tissue mammography.

- Scattered mixed densities of glandular condensations are seen involving the both breasts (ACR B).
- **A speculated border mass is seen in UOQ of left breast.**
- **A dense large left axillary LN.**
- Normal skin thickness & contour of both breasts.

High resolution breast sonography reveafd.

- Echogenic mammary parenchyma.
- **A solid hypoechoic mass with speculated borders measuring 39 X 38 mm is seen in let breast at 3 o'clock causing skin dimpling and nipple retraction.**
- No retroareolar ductal dilation.
- **Left pathological axillary LN measuring 39 X 26 mm.**
- Right likely non- specific axillary lymph nodes.

Opinion

- Left suspicious breast mass with left pathological axillary LN for biopsy (BIRADS 5).

-Right breast is free (BIRADS 1).

BIRADS calcification of breast lesions according to the American Collage of Radiology used for breast assessment combined with recommendations:

Category & recommendations	Suspected Cancer Risk
BIRADS 0: Non- informative study.	0 %
BIRADS 1: Normal, routing screening.	0 %
BIRADS 2 Benign findings, routing screening	2 %
BIRADS 3: Probably benign, for close follow up.	50 %
BIRADS 4: Probably malignant, for biopsy.	
A: mild suspicion.	
B: moderate suspicion.	
C: high suspicion	98-100%
BIRADS 5: Malignant, for biopsy.	
BIRADS 6: Pathologically proven malignant lesion.	

Signature

Dr. /SAHAREL FIKY

Consultant of Radiology

Mammogram
[[DOCTYPE:MAMMOGRAM]]

[[DOCTYPE:ECHO]]
Echo

عزة عبد الحليم عيسى

Name:

Gender: Male ☐ Female ☐ Date: 19/10

Age: 62 y/s

ECHO MEASURMENTS				
RVIDD		(7-23mm)	LAD	44
IVSD	1.0	(7-11mm)	AOR	38
LVEDD	47	(37-56mm)	EPSS	
LVESD	40	(32-40mm)	F.S	18 %
PWT	0.8	(7-11mm)	E.F	38 %

Nil

M.Mode & 2D finding			
MITRAL valve		Rt. ventricle	
Tricuspid valve		Lt.Atrium	
Aortic valve		I.V.S	
Pulmonary valve		pericardium	
Lt. vontrice		Other finding	

Doppler finding			
Mitral valve		shunt	
Aortic valve		M.V.A	
Tricuspid valve		E/A ratio	
Pulmonary valve		Other finding	

EF = 38%

Concolusion:

Moderately impaired LV Systolic Fuction

N LV internal dimsions

N LV wall Thickness

Diastolic Dysfuction grade I

Mildly Dilated LAS (Dilated LA)

Sclerotic AV with Flow

Mild MR

No PE

ICM
DD
+MR
+LA